

2025 Johns Hopkins Orthopaedic
Symposium

Narrative Medicine Program



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Navigating Orthopedic Surgical practice during Sudan's War; Challenges, Adaptations and Recommendations



Ahmed Sulieman



Awab Osman and Sayed Rasheed

Introduction: Orthopedic surgery in war-affected areas presents significant challenges that impact surgical practice and patient outcomes. Sudan, currently experiencing ongoing conflict, serves as a critical case study.

Study Design: This qualitative study involved in-depth, in-person interviews with twelve orthopaedic surgeons in Khartoum, Sudan. Focused interviews were conducted, ensuring comprehensive coverage of all the insights. The average interview duration was 45.25 (± 7.84 SD) minutes. **Objective:** To assess the challenges, adaptive strategies, and potential improvements in orthopaedic surgical care during wartime.

Methods: Surgeons' experiences were analyzed based on recurring patterns and key themes, focusing on shortages of surgical supplies, staffing issues, safety concerns, adaptive surgical techniques, and recommendations for improving care.

Results: Orthopaedic surgeons faced significant challenges due to the severe shortage of surgical supplies, leading to the reuse of instruments after sterilization and a critical lack of essential supplies such as sutures. One surgeon described, **"We were forced to reuse instruments two to three times after sterilization because we had so many patients to treat. Surgical sutures, in particular, were one of our biggest shortages."** The absence of interdisciplinary surgical staff, including vascular, neurosurgery, plastic surgery, anaesthesiologists, and nurses, further compromised care, with one surgeon stating, **"We do**

not have enough staff to provide surgical care properly, and most of those assisting were medical staff rather than surgical staff." Safety concerns were equally pressing, with hospitals becoming direct targets of violence and medical staff facing life-threatening situations. One surgeon recalled, **"I was threatened with a gun to my face multiple times. Once, they intended to shoot a doctor, but the bullet hit a security guard in the emergency department instead."** Despite these risks, surgeons adapted by utilizing unconventional tools and techniques to continue treating patients. With no access to specialized orthopaedic drills, some relied on carpenter drills, while traditional skin traction using ropes was employed for fractures. As one surgeon explained, **"For femur fractures, we use traditional skin traction in the operating room."** In cases of cervical injuries, makeshift collars were crafted from cardboard and cotton due to the lack of proper medical supplies. To improve orthopaedic care during wartime, surgeons emphasized the need for local and international collaboration to ensure the consistent supply of essential equipment and surgical staff. One surgeon highlighted, **"Most of our work is trauma-related, so we need a fully equipped trauma environment with enough personnel."** Overcrowded hospitals also posed a major challenge, as one surgeon noted, **"The capacity of this hospital cannot withstand the number of patients we receive."** Additionally, unstable salaries and unsafe working conditions led to exhaustion and emotional distress among medical professionals. One sur-

geon described, **“I was working more than 18 hours a day, treating trauma cases outside my specialty, all while having no stable salary.”**

Lastly, surgeons called for better resource allocation, pointing out inefficiencies in the distribution of essential medical supplies: **“We hear about shortages of ambulances, but they are redirected to other areas that do not need them.”**

Conclusion: This study highlights the severe

shortages of surgical resources, staffing deficits, and security risks faced by orthopaedic surgeons in Sudan. Adaptive surgical practices have helped mitigate these challenges, but long-term improvements require strategic resource allocation, infrastructure development, and international support. Addressing these challenges through targeted interventions can improve orthopaedic care in war-affected regions.



The Unknown Variable



Eve R. Glenn

Many late evenings, I find solace in my medical school's computer lab. The soft glow of my Macintosh screen creates a peaceful retreat in the dark room, letting me glimpse the brightly lit student atrium. Each unopened Safari tab holds new knowledge waiting to be discovered, enriching my understanding of medicine while serving as a gateway to the balance between research and patient care. I delve into a literature review on compartment syndrome in tibial plateau injuries, navigating the anatomy of the tibia and trauma implications. This journey bridges theoretical knowledge with patient realities. My pursuit of knowledge often conflicts with my wish for a world without injury, where patients do not endure the challenges I study. This duality weighs heavily on me, especially when unexpected post-surgical outcomes delay healing for the vulnerable.

A text from M interrupts my reflection: "How is your leg?" I ask. "A bit challenging to wake up but better. Walking. Love you," they reply. A simple exchange, yet it holds within it an entire narrative of resilience, uncertainty, and hope. Their injury, a consequence of an unassuming moment—a playful dog, a misstep—has thrust them into a new reality where autonomy is no longer assumed but fought for. I recall my studies on *Staphylococcus epidermidis*, a ubiquitous bacterium whose presence on the skin transforms into an adversary in the sterile world of surgery. But in this moment, I do not see just a pathogen; I see its impact on M—the way it shapes their fears, disrupts their ambitions, forces them into a space of dependence and vulnerability. Medicine is not merely the science of curing disease; it is the art of understanding the lived experience of illness, the negotiation of uncertainty, the emotional toll of recovery.

On my drive home, I celebrate M's resilience as they share progress in physical therapy. Our conversation reinforces a vital lesson: medical school is not just knowledge plus clinical reasoning plus professionalism. There is an additional "x" that represents the ability to stand in the liminal space between scientific certainty and human ambiguity, to bear witness to suffering with both intellectual curiosity and deep empathy. Foundational studies and technical proficiency alone are not sufficient; they must be interwoven with a conscious effort to cultivate trust, to honor the weight of each patient's story, to see beyond the diagnostic codes and into the person who carries them.

As I near the end of my preclinical training, I reflect on M and the lessons I hope to carry to the wards. I hope to integrate the technical expertise I have honed with an intentional presence—one that acknowledges the fears, aspirations, and humanity that exist beyond the confines of a patient chart. But this is not just my journey—it is ours. As future and current physicians, we all have the privilege and responsibility to balance medical expertise with human connection. In every encounter, we have the opportunity to reaffirm dignity, foster trust, and remind patients that they are seen not as conditions to be managed, but as whole individuals, carrying stories of complexity, struggle, and resilience.

Medicine is not merely about healing the body; it is about accompanying people in the intricate, often unpredictable process of recovery. It is about holding space for both science and soul, balancing the certainty of physiology with the uncertainty of human experience. And it is in this balance, this convergence of knowledge and compassion, that I hope we can all find our purpose as future and current healthcare professionals.

The Chaos I Brave



Juliane Liberto

There exists great courage
among those who venture into the chaos.

The chaos I speak of
is not of the cinematic kind,
but the quiet chaos of questions
that multiply with every new answer.

The chaos of diving deep
into an ocean of information
only to find myself drowning in it.

The chaos of wandering for days
through peer-reviewed contradictions
that leave me more lost than when I began.

The chaos of emotions
that dig deep into my soul
with every disappointed failure.

For this is the chaos I brave.

That which challenges
the boundaries of what
we think we know.

That which has the potential
to save a life,
to shift a paradigm,
to pave a new road forward.

Perhaps that is why
I chose this profession.
To seek truths not yet
accepted by our time.

Made of Fibers



Cyanotype No. 1

about Made of Fibers



Samalya Thenuwara

The background is my bilateral standing AP knee X-ray. The muscles layered on top are embellished to reflect parts of my life. The left rectus femoris is decked with sequins made to look like water lilies, a symbol of Buddhism. Next to it, the vastus lateralis is decorated with seashells to represent formative summers spent in my ancestral fishing village in Sri Lanka. Finally, the sartorius muscle is green and dotted with little red beads to look like the strawberry patch I water in my backyard. These embellishments tell a story about me: I use my legs to kneel in prayer, to run across sandy beaches, and to help my mother garden. I hoped this piece would serve as a reminder that while anatomy is recognizable and universal, it is also personal. Every body is lived in differently.

How it was made: The blue base image is a cyanotype print — a photographic process that creates deep blue images on fabric. I inverted my X-ray, digitally sketched the muscles, and laser printed the final image onto acetate. After coating the cotton fabric with light-sensitive chemicals, I placed the acetate on top and exposed it to sunlight. Once developed and the excess ink rinsed, the image appeared on the fabric. This process is finicky and unpredictable; successful prints require trial and error, especially with ink drying times and the variability of sun exposure. Finally, I hand-embroidered and beaded each muscle.

Holding Room

Sahithi Madireddy

I remembered in the months between college and medical school, Ana told me about the boy on the train tracks. I worked with Ana, a real adult, when I was in high school on a project to build youth mental health centers in my community. We were sitting in Philz, a popular local coffee spot with colorful stickers and a poster of the “7 Rules of Life” outside. The boy dedicated himself to mental health advocacy after his classmates died by suicide. A few years later—now—he graduated college. The guilt that he had made it out led him to the same railroad his classmates had stood on years ago. Ana told me during a tour of a new center that she was once pre-med but thankfully she didn’t become a doctor; “people would have died.” She was doing another kind of healing, she said.

When my summer ended, I found the preclinical curriculum strange. On the fourth day of medical school, my body donor lay naked on a table with her feet bound. On the fifth day, we started to cut into her with a scalpel. There was a Mepilex dated from July on her sacrum and a shine of polish on her nails. We saw the bruises that stained her skin and muscles dark red. It hurt to see that she was meat, the same marrow and flesh and fat. It was hard to shake how tenuous human dignity could be.

On the last day of anatomy, we heard the autopsy report that she died of respiratory complications. The clues came together: I imagined her spending her last days in a hospital bed after an untimely fall, unable to get up, her bones softening by the day. Deconditioning. She would have grown weaker, her mouth more unclean, until she aspirated some oral flora that grew in the moisture of her airways. Someone

who loved her stopped by to paint her nails. She might have been any patient in any hospital who lay in bed, every medical and nursing and physical therapy team too busy to make sure every patient ambulated every day.

We started to learn about things that would kill us. I became scared of fungal pathogens I hadn’t heard of before. I became scared of hip fractures. As I progressed through my preclinical education, death started to creep in through other places. I called a patient to enroll her in a quality improvement project for diabetes mellitus treatment, only to learn that she had passed away the previous day. I apologized and hung up. She had still been living in the medical record, not yet marked as deceased. Between a first and second trial of our intervention, I updated a spreadsheet to remove the people who died since I first spoke with them about their diabetes.

In another chart review project, I read through palliative care consults of people with cancer. There was the woman only a little bit older than me who had gastric cancer. She and her family realized in successive notes that nothing more could be done. On more than a few occasions, palliative care notes stopped the day before a patient died. They were patients even after death, briefly alive again as I was trying to piece together how they felt in their final days.

Of course, I also saw patients in front of me, and I wanted to believe I was helping them. Everyone seemed to think so. But I started to see death everywhere, and life was only the narrow negative space from which we tried to wring every bit like a wet rag. The plastic lids of takeout dinners that I ate while studying came from the

compressed bones of dinosaurs. The food that I ate came from the biochemistries of other living organisms. I worried every errant noise I heard in the night could have been the pop of a gun. I worried that the unmoving people I passed by near the hospital might have been in need of naloxone or resuscitation, and that I had failed to notice or bother or care. I worried about the small black insects that were growing in my houseplant, zipping through my room and dying in droves in the windowsill.

The world we lived in was shaped by death, and life was a holding room. I felt like a pile of mushy guts trying to take apart and figure out the strange side of life I'd found myself in. It felt arrogant to think I could do good. Despite wearing white coats, we see a muck and underbelly to human life that most do not. What we have chosen to do is hard. Sometimes I had the sense that I cared so much I stopped caring. I focused instead on being the person who could stand in the muck. Over time, I came to love that my fear could not stop me from being the person who showed up everyday.

Billions of people have died before I even knew what death was. I know there are many essays about dying, but I also recently learned most medical students do not constantly think about death. I slowly started to see behind my fear of death another truth—life was so complicated and unlikely, and beautiful, and I started to believe it too. I felt it some days for myself. I didn't have to hate that I had so many questions that were without answers; the universe was generous because I always lived in today.

Studying death is part of learning to be a doctor. I know my relationship with death will continue to evolve throughout my medical education. I cannot predict how I will feel when a patient I am caring for dies, or I lose someone I am close to in my adult life, or I see the moment someone stops breathing. Death must be something else entirely when it is the candle by

which you've led your life, the thing that made your world a little safer, or when it is your own wick getting shorter.

Perhaps the role of a physician is not to consistently ward off death. If our satisfaction rested on saving lives, we would be deeply unhappy because death happens to everyone. Much of our patients' story exists outside of us, and the events of their lives have shaped their body before they enter the hospital. The physician instead witnesses, and meets death and life as they come.

I called my best friend when she texted me that her childhood dog died. Her parents, both physicians, broke the news abruptly. I did not have much to say because I never had pets. I never met her dog and I did not love her dog. But I loved her the way a compass loves north, so I stayed on the line, responding with soft hums of acknowledgment and letting myself be with her, acting out the gestures of love. We hold room for each other, in times of emptiness and fullness, through our ideals and inconsistencies, through living and dying. There is meaning in this too.

This Barbie Is... A Dolled-Up Wax Cadaver?



Ellie Rose Mattoon

“A Replica of the Medicean Venus; with removable entrails”

I squinted at the museum placard, translated the description from German to English on Google, and squinted again.

Of the many words I could use to describe the life-sized wax woman lying down at my eye level, Venus dropped low on the list. For one, in my Percy Jackson-level knowledge of mythology, the goddess of beauty rarely shows her abdomen splayed open like a science project.

This woman is one of many Anatomical Venuses that grace the medical museums of Europe, initially commissioned as life-sized figures to teach anatomy without the hassle and smell of a rotting cadaver. When displayed in a museum, her wax glows under the overhead lights as if to let her unblemished, naked skin pop against the display case. Her breasts peel back, opened like a chapterbook to display her lungs, and a tiny stomach overflows with the organs waiting inside. Even in surgery, her eyes flutter open, long goldspun hair and a pearl necklace accentuating her neck.

Last fall, while doing research in Austria, I encountered the Anatomical Venus for the first time in Vienna’s Josephinum. One of her sisters followed me to Budapest’s Semmelweis Museum,

and others beckoned me to visit them in Florence’s La Specola. The Anatomical Venus has male counterparts—most of them stand fully erect and skinned, showing off how their muscles bulge in motion. However, they didn’t seem to capture as much intrigue from myself or my fellow museum visitors. The Anatomical Venus lays on a velvet bed though, welcoming us to take our time and — thanks to her removable organs — take her apart.

Where in the cultural zeitgeist of 18th-century Europe had this woman sprung? And to what extent would she still follow me from gross anatomy class to clinical rounds?

It’s hard to put my reaction to the Anatomical Venus into a single word. Shock, possibly. Yet shock can be both positive and negative. Disappointment that a life-sized Barbie used to teach female anatomy—my anatomy? Maybe, though I don’t think Barbie is a fair comparison. Maybe I even felt a small sense of pride. I’m starting medical school this summer, and this woman will never get off her reclining couch.

Despite my doubts, I couldn’t stop thinking about Venus long after I left the Josephinum. It was as if she followed me back to New York, languidly taking up three seats in economy class, and begging me to look at her some more.

Medicine's perception of the female body did not start with the Anatomical Venus. However, she feels like a crucial lamppost in the history that brings us to 2024's less-than-ideal state of women's health. Despite the NIH requirement that women be included in federally-funded research, a recent analysis shows that they are still underrepresented in medical trials involving everything from treating cancer to heart problems. A 2020 article reported that searching "semen" on PubMed returns almost thirty-eight times more results than a search for "menstrual blood." Women of color and pregnant women face even more severe underrepresentation in research, which may be part of the reason that Black women are 2.6 times more likely to die from pregnancy-related complications than white women.

Perhaps I spoke too soon when I said the Anatomical Venus filled me with pride. What if we haven't come as far as I thought we had?

While many today consider the discipline a science, anatomy has a long-standing reliance on the field of art. How else could the visual information of a person's insides be communicated in the era before the camera? Yet art has a tendency to sop up a culture—its values and biases meshed together—much better than it manages to depict reality.

In the exploration of anatomy, artists and doctors shared a sort of symbiosis. Artists relied on doctors for access to cadavers, while doctors relied on artists to accurately depict the anatomy these cadavers taught. Renaissance sculptor Michelangelo engaged in anatomical dissections that allowed him to create the "heroic male" musculature in his David statue (or even in his female Libyan Sybil). Da Vinci also engaged in dissections that informed his construction of the Vitruvian Man, a perfectly proportioned human figure.

It seems only fair to ask: why didn't Da Vinci create a Vitruvian woman?

Da Vinci at least tried to study female bodies;

what we read in medical studies, is often very much framed by a particular lens," she adds.

Hill-Edgar is in many ways a modern version of the artist-anatomist. After completing medical school and her internship in internal medicine at Columbia, she received her Master of Fine Arts degree in painting from the New York Academy of Art and has shown her work in multiple solo and group exhibitions.

Yet nearly five hundred years after Vesalius and Da Vinci, she has still noted ways in which the female body — the body of approximately half the world — is considered a deviation.

Even though skeletons are often used to symbolize the universality of the human race, few people realize that male and female skeletons are markedly different. In 2021, Hill-Edgar needed a female skeleton for one of her art projects and explored the website of Anatomy Warehouse, a company that sells teaching skeletons to schools and hospitals. While the company offered countless options for a "universal" (or male) skeleton, it was only after pages of searching that Hill-Edgar found a female skeleton.



his 1508 study of female anatomy came pretty close. Of course, he also based several female organs on animal ones, and he carried a strange belief in a nonexistent vein that connected the ovaries to the breasts.

One explanation for these shortcomings was that female corpses were rare. Executed prisoners were a significant source of anatomical cadavers; not only were women the minority of this demographic, but they couldn't be executed if pregnant.

Besides the paucity of cadavers, a second explanation lies in culture. In both art and anatomy, male bodies were considered the universal prototype, females the aberration or "reproductive other." From prehistoric times, the earliest depictions of female bodies exaggerate their reproductive organs, and their fertility, above all else. Even the Venus de Medici, on which her anatomical sister is based, is a 100 BC depiction of the fertility goddess from Greece.

This isn't of course to say that any piece of art tradition is objectively good or bad, beautiful or preposterous. Yet it's easy to look at this pattern and see how it seeped into early depictions of the female body, those that came before the Anatomical Venus. It explains why, for example, in a 1541 anatomy book, the male figure is shown completely dissected, the female figure only torn apart around her reproductive organs.

"In medicine, things are often presented as objective or fact. In art history, one considers more of a subjective or cultural experience," says Allison Hill-Edgar, an artist, medical doctor, and independent scholar.

As a DeBakey Fellow at the National Library of Medicine, Hill-Edgar has been researching representations of female anatomy in medical images and how that impacts medicine today.

"What we see in factual medical documents,

The least expensive female skeleton cost \$1,200 in comparison to the male skeleton's \$189 (a 2024 revisit of the same site found that the female skeleton was \$1305 while the male was \$249).

"If we see a male skeleton, that is the skeleton."

When she contacted the company, eventually having a conversation with the director, he noted to her that the pricing was a matter of supply and demand. Hill-Edgar eventually relented and bought the expensive skeleton out of necessity, and Anatomy Warehouse sent her an extra bag of assorted female bones as consolation.

"If women's bodies are still being treated as such a separate category that's so much less frequent that it needs to be priced five times as much, yeah, we're never going to have representation."

The Anatomical Venus came about in the late 1700s by the command of Leopold II, who commissioned wax artist Clemente Susini to create life-sized anatomical models for Florence's first public science museum — La Specola. Susini attempted to accurately depict the internal anatomy from real-life dissections prepared for the commissioned wax artists, but he also took artistic liberty in preparing his figures' poses.

At the time, many visitors to Florence were wealthy European aristocrats embarking on their "Grand Tour," the Enlightenment equivalent of a study-abroad bender across the continent. Florence was the city of art, and the fertility goddess Venus was a long-standing symbol of the city. Visitors already flocked to Titian's Venus d'Urbino, Botticelli's Birth of Venus, and the aforementioned Venus de Medici. Why not make them flock to one more Venus, this one a teaching tool? In many ways, the Anatomical Venus was an 18th-century take on science communication.

"La Specola was the first museum open to everyone, accepting visitors from any social class," says Roberta Ballestriero, lecturer at the Academy of

Fine Arts of Venice. “From the aristocracy to the working class.”

In an interview, she explained to me why the artists bothered giving some of the Anatomical Venuses long flowing hair and jewelry. In some instances, these accessories covered a scar on the model’s neck, the cut of her removable abdomen. Felice Fontana, La Specola’s museum director, insisted that the models be unblemished. The public wouldn’t typically want to look at a decaying body, but maybe they would be forced or compelled to look at something that invited them to.

“To see nudity in Britain might have been shocking. In Florence, Italy, it’s the air you’re breathing.”

“More than saying if there were beautiful craft-work or not, it is important to remember that

they were useful didactic models,” Ballestriero says. “They were helping ordinary people understand some anatomy, especially those with no knowledge or resources.”

Joanna Ebenstein, author of *The Anatomical Venus: Wax, God, Death & the Ecstatic*, now directs the Morbid Anatomy Library, a modern-day Wunderkammer nestled in Brooklyn’s Industry City and open to the public. Months after seeing the Anatomical Venus for the first time, I visited the space to learn more.

Ebenstein explained that my contemporary reaction to the wax figure was likely different from that of the model’s target audience. Guestbooks from La Specola mostly speak of visitors in awe, not people shocked or disgusted by her nudity.

“The place where it crosses a boundary is the fact that it’s supposed to be a medical representation, which in our day and age is considered an objective one. It’s clearly not objective.”

Both agree that, even if the Anatomical Venuses started as a teaching tool, they later became sexualized. Ballestriero notes that copies of her went on tour across Europe as the centerpiece of fairground exhibitions, some of which were men’s-only. In response to one such exhibition in London, the *Literary Gazette* called the Anatomical Venus “a filthy French figure” and “as indecent as it is wretched.”

Hill-Edgar gave her own comment on the Anatomical Venus, acknowledging that even though much artistic representation of women historically has been sexualized, that doesn’t make it right when it appears in medicine.

“At the time, they were exploratory models, you were supposed to take out the parts, and the ultimate place that you got to was the open uterus with the fetus inside,” she explained. “So there was this whole tactile exploration of a dead woman’s body who was sexually alive.”



“I feel it’s really about your personal response,” Ebenstein said in our interview. “I find it still a beautiful and complicated thing. And I also very very strongly feel that complicated things should be shown.”

Growing up in Austin, Texas, I did not learn my anatomy basics from a museum. Instead, in the sixth grade, I checked out a medical encyclopedia from the library and resolved to read it from A to Z. A curly-haired girl who saw me lugging the opus against my chest from class to class asked if that meant I wanted to be a doctor. I remember that was the first time anyone had asked me that question.

While I didn’t retain much from my week-long dive from “Aarskog syndrome” to “Zollinger-Elison,” it’s unlikely that I saw many women inside that encyclopedia. As late as 1986, an analysis of anatomical textbooks showed that only 11% of gendered images outside of the chapter detailing the reproductive system were of women. Never mind that women have different skeletons, different manifestations of cardiac disease, and different rates of drug metabolism. The percentage of female images among gendered images in anatomy textbooks rose to an average of 32% in a 1994 analysis, then crept to 36% in a 2017 one.

When gendered images are shown, the pattern remains similar to what a visitor might have seen in La Specola: A typically white “muscle man” depicts the majority of “normal” anatomy, while a young, toned woman depicts her re-

productive system. Some images of women from above or from a caudal (camera between the legs) perspective remind me a bit too much of some of the Anatomical Venus’ descendants, some of which include white-cuffed, phantom hands that pry open a surgical incision or help a viewer see more of the female genitalia. It’s perhaps not a surprise that these biased images have been shown to implicitly shape our gender biases.

“Not considering women or different races as equal parts of the fabric of medicine is leading to really disastrous outcomes.”

Even if there were female depictions, any anatomical depictions of people of color, male or female, are rare. A 2022 German analysis of three textbooks, six atlases, and two e-learning platforms found that only three illustrations — approximately one in 100,000 images — showed a non-white person. Never mind that skin diseases may appear differently on people of color or that different racial or ethnic communities may have genetic risk factors for diseases such as sickle cell anemia.

Hill-Edgar has noted in her research that, in the history of female anatomy, bodies of women of color were often treated with even lower ethical standards than their white counterparts. Enslaved women were subject to hypersexualization and unconsented examination in the name of medical study.

Take Sarah Baartman for example. Her adult life in the 1810s was spent traveling Europe with her enslaver under the derogatory moniker “the Hottentot Venus.” For a price, spectators ogled her buttocks (unusually large due to a genetic condition) and subjected her to various forms of sexual abuse. After her death, her genitals remained on display at Paris’ Museum of Man until 1974. Her remains were not returned to her hometown in South Africa until 2002.

“Not considering women or different races as



equal parts of the fabric of medicine is leading to really disastrous outcomes,” she said.

Medicine prides itself on its long tradition of building on the past, and in visiting all these medical museums I felt tempted to build a strange solidarity with the (mostly) men who learned from these materials and used these (now-rusty) surgical instruments on real people. If I met these medical students of the past, could we bond over the human body and the infinity of miracles that keep it alive?

Yet as I wandered through these museums, the Anatomical Venus always lingered as a sort of awakening, reminding me that I am not a medical student of the past, nor will I ever be. If I met the medical students of the past, they might just wonder how a living woman got inside the anatomy theatre.

Hill-Edgar shared her own complicated feelings about her role in the art-anatomy tradition.

“Even more than an affinity with the artist-anatomists, at this point, I feel a very strong affinity with the female bodies,” she says.

“As a woman, when you see some of these things, you try to understand: who was this person? Why her body? Why was it represented this way? I definitely feel like, in many ways, those women are the ones inspiring me.”

If art does tend to sop up an era, there’s still hope for the creators of today to modernize anatomy. For example, Chidiebere Ibe, a Nigerian medical student at Copperbelt University in Zambia and a medical illustrator with Harvard, focuses his portfolio on illustrations that depict people of color.

In 2021, Ibe posted one of his illustrations of a pregnant Black woman and her fetus with the caption “I’m black and black is beautiful!” Ibe says that this is the first image he’s seen of a Black fetus in utero.



“I had a woman who literally called me in tears because they felt represented,” he said.

Ibe is currently running a training program at Copperbelt University for medical students to become medical illustrators, and last June, he participated in the launch of Illustrate Change by Johnson & Johnson, an online anatomy library of racially diverse illustrations.

I asked Ibe the same question I had asked Hill-Edgar: do you feel a solidarity with the artist-anatomists of the past, or do you feel like you are charting your own way?

“To create something for the future, we need to reflect on the past. Right?” he returned the question to me.

When I ponder this exchange, in many ways this answer is as grey as the Anatomical Venus herself: neither dead nor alive, neither human nor doll. Her world hides dark pockets that demand we do better, but it also led me to people bringing this history forward. While I might not want to go back and say hello to every medic of the past, I too can be who I want to be for the students of the future.

“How can something that makes you want to know more about its history — how can that be bad?”

At the end of the day, the Anatomical Venus was designed to make me look. And she did her job, taking me from Vienna to Brooklyn with calls to Nigeria, through thousands of years of history to questions about my own future. Not bad for a Barbie I initially criticized for not being able to get off the couch.

In my conversation with Ebenstein, she shared a similar sentiment about her relationship with the Anatomical Venus.

“I feel like I could spend the rest of my life going down all the paths that lead from that and not have enough time to finish,” she said.

“How can something that makes you want to know more about its history — how can that be bad?”

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